PROJECT PLAN

Identity and Access Management (IAM) System Deployment for Cybersecurity Enhancement

Corella Health Group (case organisation), regional New South Wales

Version 1.0 · 25 October 2026

Team member	Project role	Plan sections owned
Toufiq Ahmed Joy	Project Manager (team lead)	1, 3, 4, 5, 6, 8
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Project sponsor: Catherine Doyle, Chief Operating Officer, Corella Health Group

Document control

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Approval	Catherine Doyle, Chief Operating Officer (project sponsor)
Source files	GitHub repository https://github.com/AR13X3/iam-deployment-project-plan; schedule, WBS, risk register and templates are kept there under version control (Section 11).
Case note	Corella Health Group, its people and its audit figures are a hypothetical case organisation used for this plan. All other facts are cited.

1. Introduction 3

2. Background research 3

2.1 Why identity, and why now 3

2.2 How strong MFA needs to be 3

2.3 Usability and the access model in clinical settings 3

2.4 Project-management practice 4

3. Project overview and objectives 4

4. Project charter and scope statement 4

4.1 Charter summary 4

4.2 Scope 5

4.3 Stakeholders 5

4.4 Project team 6

4.5 Milestones and gates 6

5. Work breakdown structure 6

6. Project schedule (Gantt chart) 9

7. Resource allocation and responsibility matrix 12

8. Risk management plan 13

9. Issue management strategy 16

10. Change request and change management process 16

11. Configuration management (GitHub) 18

12. Communication management plan 21

13. Quality management approach 22

14. Conclusion 23

References 23

Appendix A. Jira project evidence 25

Appendix B. GitHub history 27

Appendix C. Working calendar 27

1. Introduction

Section owner: Toufiq Ahmed Joy

Corella Health Group (CHG) is a not-for-profit healthcare provider in regional New South Wales. It has 1,180 staff and about 320 contractors and visiting clinicians across 9 sites. They use 42 applications, including an electronic medical record (EMR) and a patient administration system (PAS).

Problem statement. CHG cannot reliably control who has access to what: every application keeps its own accounts, managed by hand. A 2026 audit found 186 active accounts of people who had left, 38 administrator accounts where about 12 are needed, a 6.5-day wait for new-starter access, and multi-factor authentication (MFA) used by only 31% of staff. A phished password later caused a notifiable data breach. Credentials were stolen in a quarter of healthcare breaches worldwide [1], and Australian health providers made the most breach notifications of any sector in 2025 [2].

The project. CHG will deploy a central identity and access management (IAM) system: single sign-on (SSO), phishing-resistant MFA for every user, access driven by the HR system, role-based access, quarterly access reviews and privileged access management. It runs from 2 November 2026 to 28 July 2027 within a budget ceiling of AUD 1,250,000. Sections 2–14 follow the contents list.

2. Background research

Section owner: Fardin Rahman Tanvir

2.1 Why identity, and why now

Worldwide, exploited software flaws (31%) now outrank stolen credentials (13%) as the first step in a breach, but credential abuse is still 25% in Asia-Pacific and credentials were taken in 25% of healthcare breaches [1]. In Australia, phishing, stolen credentials and brute force caused 151 of 247 cyber incidents in July–December 2024, with health the worst-hit sector [3]. **For this plan:** identity is a major exposure but not the only one, so patching stays with CHG's Essential Eight program and risk R09 covers attacks during the transition.

2.2 How strong MFA needs to be

All sources agree any MFA beats none: in Microsoft's data it cut account takeover by 99.22% [4], though that figure is vendor data and not peer-reviewed. Government sources set a higher bar: CISA calls SMS codes a last resort [5], NIST restricts SMS [6], and the Essential Eight requires phishing-resistant MFA at Maturity Level Two [7]. With 14.2% of nearly three million test phishing emails clicked at US health institutions [8], codes can be stolen. **For this plan:** phishing-resistant MFA for all users (passkeys, or security keys at shared workstations), SMS only by approved exception, administrators and HR first.

2.3 Usability and the access model in clinical settings

Controls that slow clinicians lead to workarounds: in compulsory two-factor rollouts more than one attempt in twenty failed, worse with many separate log-ins [9]. Health-record studies favour attribute-based rules but find gaps in MFA, emergency access and audit trails [10]. NIST also bases access on identity plus attributes and advises migrating one process at a time, report-only first [11]. No source requires quarterly reviews; the Essential Eight asks for yearly checks of administrator access [7]. **For this plan:** SSO, tap-and-go keys on ward computers, roles from HR job codes with site and shift rules, monitored break-glass accounts, report-only testing, rollout in waves, and quarterly reviews chosen for the risk.

2.4 Project-management practice

The PMBOK Guide (8th edition) expects each project to tailor its methods [12], and ISO 21502 fits any delivery approach [13]; neither picks one. ISO 31000 gives a general risk process [14], but risk management adds value only if people see and use it [15]. **For this plan:** a hybrid approach, with gates, baselines and a change control board for contract and compliance work, iterative SSO waves and a pilot, and clinical and IT staff in risk reviews, every risk with an owner and trigger.

3. Project overview and objectives

Section owner: Toufiq Ahmed Joy

The project replaces account management application by application with one identity platform fed by the HR system. When People and Culture records a hire, move or exit, the platform creates, changes or removes access in connected applications using role-based rules. Staff sign in once with phishing-resistant MFA; administrators use separate privileged accounts; managers re-certify their staff's access every quarter. The business case is to close the audit findings, lower the likelihood and cost of another breach, end the wait for new-starter access, cut password-reset calls and reach Essential Eight Maturity Level Two for MFA and restricting administrative privileges [7], which the board has set as its target for 30 June 2027. Table 1 lists the measurable objectives.

Table 1. SMART objectives

ID	Objective	Measure and target	Date
O1	Phishing-resistant MFA for every identity	100% of 1,500 in-scope identities enrolled and enforced (passkeys or FIDO2 keys); SMS used only by approved exception	16 Jun 2027
O2	Single sign-on	12 priority applications sign in through the IAM platform	16 Jun 2027
O3	Fast, reliable leavers	Access to all in-scope applications removed within 4 hours of the HR termination record, in a monthly sample of 30 leavers	From 16 Jun 2027; first check 14 Jul 2027
O4	Day-one access	At least 95% of new starters have role-based access before their first shift (currently a 6.5-day wait)	Measured over the 3 months to 30 Sep 2027
O5	Account clean-up	All 186 orphaned accounts disabled; administrator accounts cut from 38 to 12 or fewer	28 Jul 2027
O6	Access reviews	First quarterly review covers 100% of entitlements in the 12 applications, with every revocation actioned	14 Jul 2027
O7	On time and on budget	Project closed within the AUD 1,250,000 budget ceiling	28 Jul 2027

4. Project charter and scope statement

Section owner: Toufiq Ahmed Joy

4.1 Charter summary

Table 2. Project charter

Project title	Identity and Access Management (IAM) System Deployment for Cybersecurity Enhancement
Organisation	Corella Health Group (CHG), regional New South Wales
Project sponsor	Catherine Doyle, Chief Operating Officer
Project manager	Toufiq Ahmed Joy
Purpose	Give every staff member and contractor the right access at the right time, protected by phishing-resistant MFA, and remove access promptly when it is no longer needed.

Start and finish	2 Nov 2026 to 28 Jul 2027 (182 working days)
Budget	Ceiling AUD 1,250,000: cost baseline AUD 1,199,030 (including AUD 159,000 contingency reserve) plus AUD 50,970 management reserve held by the sponsor. Excludes GST.
Authority	The project manager may commit funds within approved work packages and approve changes within the limits in Section 10.

4.2 Scope

Table 3 shows what is in and out of scope. Anything not listed as in scope is out unless the change control board adds it (Section 10). Table 4 lists the deliverables, assumptions, constraints and acceptance criteria.

Table 3. In-scope and out-of-scope items

In scope	Out of scope
• 1,500 workforce identities: 1,180 staff and 320 contractors, agency staff and visiting medical officers • 12 applications: Microsoft 365, intranet, VPN, rostering, EMR, PAS, HR system (HRIS), payroll, finance, pathology results portal, imaging viewer, incident management • MFA for all users; privileged access management for 40 privileged accounts • Joiner-mover-leaver automation from the HRIS; role catalogue of about 25 business roles • Quarterly access reviews; training at all 9 sites	• Patient and consumer identities (patient portal) • The other 30 lower-risk applications (phase 2 backlog) • Medical devices and building access cards • Replacing the HRIS, EMR or any other application • Endpoint management, patching and email security (Essential Eight program) • Identities for partner organisations other than contractors

Table 4. Deliverables, assumptions, constraints and acceptance criteria

Deliverables	Configured IAM platform; HR-driven joiner-mover-leaver workflows; SSO for 12 applications; phishing-resistant MFA for all users; role catalogue; privileged access management; access review campaigns; trained staff and support materials; runbooks and handover; the project documents in this plan
Assumptions	The HRIS is the authoritative record of staff and contractors; CHG's Microsoft 365 tenant can be federated with the IAM platform; application vendors support integration under standard support; clinical testers are available with funded backfill; the implementation partner is engaged at M3
Constraints	Budget ceiling AUD 1,250,000; go-live before 30 June 2027; no clinical system cut-overs in business hours; identity data stored in Australia; change freeze during the Christmas shutdown (24 December to 1 January)
Acceptance criteria	Objectives O1–O6 met and evidenced; no open severity 1–2 defects at Gate 3; all high or critical penetration-test findings fixed; runbooks accepted by the IT Operations Manager; closure report signed by the sponsor

4.3 Stakeholders

Table 5. Stakeholder register (summary)

Stakeholder	Interest	Influence	Needs from the project
Project sponsor: Catherine Doyle, Chief Operating Officer	Close audit findings; meet the board's Essential Eight target	High	Early escalation; status against baseline
Executive leadership team and Board Audit and Risk Committee	Cyber risk, compliance, budget	High	Evidence of risk reduction; budget kept
Head of Information Technology	Owns the platform after handover	High	Supportable design; runbooks; trained staff
IT operations and service desk	Run sign-in and provisioning day to day	Medium	Knowledge articles; rollout dates; staffing at go-live
Clinical staff and the Director of Nursing	Fast, reliable access at the bedside	High	Tap-and-go sign-in; no disruption to care; training on shift

Stakeholder	Interest	Influence	Needs from the project
Administrative and corporate staff	Simpler sign-in	Medium	One sign-in; clear instructions
Contractors, agency staff and visiting medical officers	Access ready on first shift	Low	Simple enrolment and access requests
People and Culture (HR)	HRIS becomes the trigger for access	High	Data rules and process changes agreed
Privacy Officer and compliance	Privacy and health-record obligations	Medium	Privacy impact assessment; Australian data residency; audit evidence
IAM platform vendor and implementation partner	Deliver the contract on time	Medium	Clear requirements; quick decisions; system access
Application owners (EMR, PAS, payroll, finance and others)	Integration without breaking their systems	Medium	Test windows; cut-over and rollback plans

4.4 Project team

Table 6. Project team roles and responsibilities

Name	Project role	Responsibilities
Toufiq Ahmed Joy	Project Manager (team lead)	Leads planning and delivery; owns the charter, scope, WBS, schedule and risk register; reports to the sponsor; presents change requests to the CCB
Kritika Bhattarai	Resource and Communications Manager	Resource plan and RACI matrix; issue log; stakeholder communication, training and the clinical champions network
Fardin Rahman Tanvir	Quality Assurance Lead	Requirements traceability; test strategy, system and user acceptance testing; quality metrics and audits; research and references
Ahmed Mahir Daiyan	Change and Configuration Manager	Change control (CCB secretary); GitHub configuration management; pilot and rollout coordination; handover to operations

Other project roles (IAM Architect, implementation partner, HR systems analyst, Privacy Officer and service desk) are allocated in Section 7.

4.5 Milestones and gates

Table 7. Milestones (dates match the Gantt chart in Section 6)

ID	Milestone	Date	Exit criteria (approved by sponsor)
M1	Charter approved	6 Nov 2026	Charter signed
M2	Plan baselined (Gate 1)	24 Nov 2026	Plan, schedule and cost baselines approved
M3	Platform contract signed	23 Dec 2026	Contract signed after privacy impact assessment
M4	Design approved (Gate 2)	15 Jan 2027	Design, role model and privacy impact assessment approved
M5	Go-live readiness (Gate 3)	29 Apr 2027	Testing passed; no open severity 1–2 defects; high or critical penetration-test findings fixed
M6	Go-live complete	16 Jun 2027	All 9 sites live; MFA enforced for all users
M7	Project closed	28 Jul 2027	Handover accepted; closure report signed

5. Work breakdown structure

Section owner: Toufiq Ahmed Joy

Figure 1 shows the WBS. It has seven level-2 elements covering the whole life cycle, from planning to closure, with project management as its own branch.

- **29 work packages.** Each produces one deliverable, has one owner and appears on the Gantt chart under the same code.
- **Size.** Each work package takes 4–20 working days, so progress can be reported weekly.
- **Exception.** Package 1.1.4 covers ongoing monitoring and control from Gate 1 to the end of hypercare.
- **Milestones.** M1–M7 are approval points with zero duration, not work packages.

A text version is in wbs/wbs.csv.

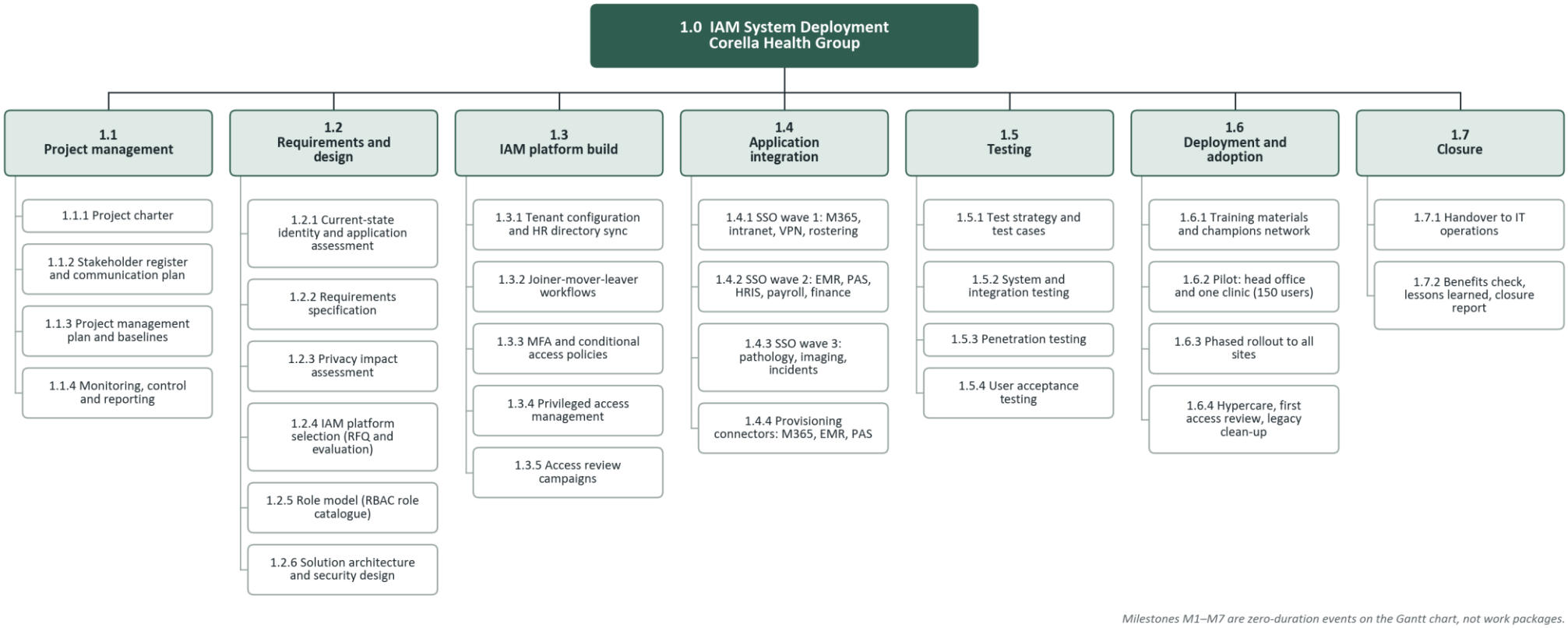


Figure 1. Work breakdown structure: 7 level-2 elements and 29 work packages

6. Project schedule (Gantt chart)

Section owner: Toufiq Ahmed Joy

The schedule was built from the WBS. It uses a five-day working week that leaves out NSW public holidays and CHG's Christmas shutdown (Appendix C), and durations are in working days.

Figure 2 is the Gantt chart. It shows every work package, every dependency link and the seven milestones, with the critical path in red.

The project starts on 2 November 2026, goes live on 16 June 2027 and closes on 28 July 2027. Go-live is before the board's 30 June target, and the close date matches the charter.

Critical path. The 182-day critical path runs through requirements, platform selection, design, MFA policies, the three SSO waves, system and user acceptance testing, pilot, rollout, hypercare and closure; platform selection and SSO wave 2 (EMR and PAS) are watched most closely. Table 8 explains the key dependencies. Test preparation (43 days) and training materials (37 days) have the most float; penetration testing has only 2, so it is managed as near-critical.

Table 8. Key dependencies and why they exist

Link	Type	Reason
1.2.4 evaluation and 1.2.3 privacy impact assessment → M3 contract	Finish-to-start	Data residency is settled before the contract is signed
1.3.3 MFA policies → 1.4.1 SSO wave 1 → waves 2 and 3	Finish-to-start	No application moves to SSO without MFA in place
1.4.3, 1.4.4, 1.3.4, 1.3.5, 1.5.1 → 1.5.2 system testing	Finish-to-start	Testing starts only when every build and integration package is done
1.3.2 joiner-mover-leaver → 1.4.4 provisioning connectors	Start-to-start + 5 days	Connectors reuse the attribute mappings defined in the first week of 1.3.2
1.6.2 pilot → 1.6.3 rollout	Finish-to-start + 3 days	Leaves time for the go/no-go review after the pilot

Table 9, after the chart, lists each task's dates, predecessors, float and resources.

WBS	Task	Days	Finish
1.1	**Project management**		**14 Jul 27**
1.1.1	Project charter	5	06 Nov 26
M1	**Charter approved**	0	06 Nov 26
1.1.2	Stakeholder register and communication plan	4	12 Nov 26
1.1.3	Project management plan and baselines	8	24 Nov 26
M2	**Plan baselined (Gate 1)**	0	24 Nov 26
1.1.4	Monitoring, control and reporting	155	14 Jul 27
1.2	**Requirements and design**		**15 Jan 27**
1.2.1	Current-state identity and application assess…	10	20 Nov 26
1.2.2	Requirements specification	8	02 Dec 26
1.2.3	Privacy impact assessment	8	14 Dec 26
1.2.4	IAM platform selection (RFQ and evaluation)	15	23 Dec 26
M3	**Platform contract signed**	0	23 Dec 26
1.2.5	Role model (RBAC role catalogue)	12	18 Dec 26
1.2.6	Solution architecture and security design	10	15 Jan 27
M4	**Design approved (Gate 2)**	0	15 Jan 27
1.3	**IAM platform build**		**01 Mar 27**
1.3.1	Tenant configuration and HR directory sync	8	28 Jan 27
1.3.2	Joiner-mover-leaver workflows	15	18 Feb 27
1.3.3	MFA and conditional access policies	6	05 Feb 27
1.3.4	Privileged access management	10	19 Feb 27
1.3.5	Access review campaigns	6	01 Mar 27
1.4	**Application integration**		**30 Mar 27**
1.4.1	SSO wave 1: M365, intranet, VPN, rostering	10	19 Feb 27
1.4.2	SSO wave 2: EMR, PAS, HRIS, payroll, finance	15	12 Mar 27
1.4.3	SSO wave 3: pathology, imaging, incidents	10	30 Mar 27
1.4.4	Provisioning connectors: M365, EMR, PAS	12	22 Feb 27
1.5	**Testing**		**29 Apr 27**
1.5.1	Test strategy and test cases	6	25 Jan 27
1.5.2	System and integration testing	12	15 Apr 27
1.5.3	Penetration testing	8	27 Apr 27
1.5.4	User acceptance testing	10	29 Apr 27
M5	**Go-live readiness (Gate 3)**	0	29 Apr 27
1.6	**Deployment and adoption**		**14 Jul 27**
1.6.1	Training materials and champions network	10	05 Mar 27
1.6.2	Pilot: head office and one clinic (150 users)	10	13 May 27
1.6.3	Phased rollout to all sites	20	16 Jun 27
M6	**Go-live complete**	0	16 Jun 27
1.6.4	Hypercare, first access review, legacy clean-up	20	14 Jul 27
1.7	**Closure**		**28 Jul 27**
1.7.1	Handover to IT operations	5	21 Jul 27
1.7.2	Benefits check, lessons learned, closure report	5	28 Jul 27
M7	**Project closed**	0	28 Jul 27

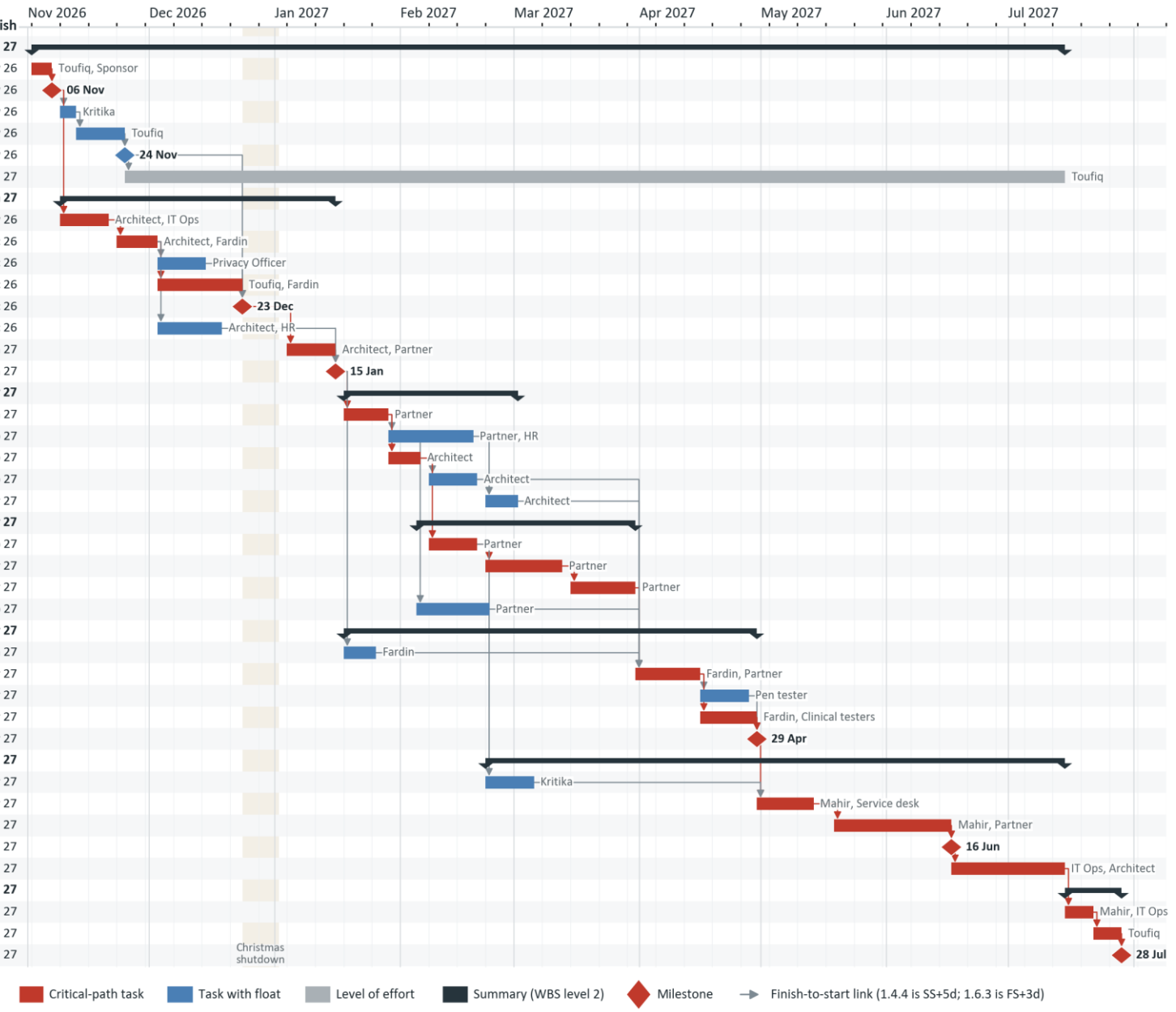


Figure 2. Gantt chart, 2 Nov 2026 to 28 Jul 2027, with dependency links, milestones and the critical path (red). Source: schedule/IAM_Schedule.gan

Table 9. Schedule task list (WBS order). Float 0 = critical path

WBS	Task	Days	Start	Finish	Predecessors	Float	Resources
1.1	**Project management**		**2 Nov 2026**	**14 Jul 2027**			
1.1.1	Project charter	5	2 Nov 2026	6 Nov 2026	-	0	Toufiq, Sponsor
M1	**Charter approved**	0	6 Nov 2026	6 Nov 2026	1.1.1	0	Sponsor
1.1.2	Stakeholder register and communication plan	4	9 Nov 2026	12 Nov 2026	M1	10	Kritika
1.1.3	Project management plan and baselines	8	13 Nov 2026	24 Nov 2026	1.1.2	10	Toufiq
M2	**Plan baselined (Gate 1)**	0	24 Nov 2026	24 Nov 2026	1.1.3	10	Sponsor
1.1.4	Monitoring, control and reporting (level of effort)	155	25 Nov 2026	14 Jul 2027	M2	–	Toufiq
1.2	**Requirements and design**		**9 Nov 2026**	**15 Jan 2027**			
1.2.1	Current-state identity and application assessment	10	9 Nov 2026	20 Nov 2026	M1	0	Architect, IT Ops
1.2.2	Requirements specification	8	23 Nov 2026	2 Dec 2026	1.2.1	0	Architect, Fardin
1.2.3	Privacy impact assessment	8	3 Dec 2026	14 Dec 2026	1.2.2	7	Privacy Officer
1.2.4	IAM platform selection (RFQ and evaluation)	15	3 Dec 2026	23 Dec 2026	1.2.2	0	Toufiq, Fardin
M3	**Platform contract signed**	0	23 Dec 2026	23 Dec 2026	1.2.4, 1.2.3, M2	0	Sponsor
1.2.5	Role model (RBAC role catalogue)	12	3 Dec 2026	18 Dec 2026	1.2.2	13	Architect, HR
1.2.6	Solution architecture and security design	10	4 Jan 2027	15 Jan 2027	M3	0	Architect, Partner
M4	**Design approved (Gate 2)**	0	15 Jan 2027	15 Jan 2027	1.2.6, 1.2.5	0	Sponsor
1.3	**IAM platform build**		**18 Jan 2027**	**1 Mar 2027**			
1.3.1	Tenant configuration and HR directory sync	8	18 Jan 2027	28 Jan 2027	M4	0	Partner
1.3.2	Joiner-mover-leaver workflows	15	29 Jan 2027	18 Feb 2027	1.3.1	20	Partner, HR
1.3.3	MFA and conditional access policies	6	29 Jan 2027	5 Feb 2027	1.3.1	0	Architect
1.3.4	Privileged access management	10	8 Feb 2027	19 Feb 2027	1.3.3	19	Architect
1.3.5	Access review campaigns	6	22 Feb 2027	1 Mar 2027	1.3.2, 1.3.4	19	Architect
1.4	**Application integration**		**5 Feb 2027**	**30 Mar 2027**			
1.4.1	SSO wave 1: M365, intranet, VPN, rostering	10	8 Feb 2027	19 Feb 2027	1.3.3	0	Partner
1.4.2	SSO wave 2: EMR, PAS, HRIS, payroll, finance	15	22 Feb 2027	12 Mar 2027	1.4.1	0	Partner
1.4.3	SSO wave 3: pathology, imaging, incidents	10	15 Mar 2027	30 Mar 2027	1.4.2	0	Partner
1.4.4	Provisioning connectors: M365, EMR, PAS	12	5 Feb 2027	22 Feb 2027	1.3.2 SS+5d	24	Partner
1.5	**Testing**		**18 Jan 2027**	**29 Apr 2027**			
1.5.1	Test strategy and test cases	6	18 Jan 2027	25 Jan 2027	M4	43	Fardin
1.5.2	System and integration testing	12	31 Mar 2027	15 Apr 2027	1.4.3, 1.4.4, 1.3.4, 1.3.5, 1.5.1	0	Fardin, Partner
1.5.3	Penetration testing	8	16 Apr 2027	27 Apr 2027	1.5.2	2	Pen tester
1.5.4	User acceptance testing	10	16 Apr 2027	29 Apr 2027	1.5.2	0	Fardin, Clinical testers
M5	**Go-live readiness (Gate 3)**	0	29 Apr 2027	29 Apr 2027	1.5.3, 1.5.4	0	Sponsor
1.6	**Deployment and adoption**		**22 Feb 2027**	**14 Jul 2027**			
1.6.1	Training materials and champions network	10	22 Feb 2027	5 Mar 2027	1.4.1	37	Kritika
1.6.2	Pilot: head office and one clinic (150 users)	10	30 Apr 2027	13 May 2027	M5, 1.6.1	0	Mahir, Service desk
1.6.3	Phased rollout to all sites	20	19 May 2027	16 Jun 2027	1.6.2 FS+3d	0	Mahir, Partner
M6	**Go-live complete**	0	16 Jun 2027	16 Jun 2027	1.6.3	0	Sponsor

WBS	Task	Days	Start	Finish	Predecessors	Float	Resources
1.6.4	Hypercare, first access review, legacy clean-up	20	17 Jun 2027	14 Jul 2027	M6	0	IT Ops, Architect
1.7	**Closure**		**15 Jul 2027**	**28 Jul 2027**			
1.7.1	Handover to IT operations	5	15 Jul 2027	21 Jul 2027	1.6.4	0	Mahir, IT Ops
1.7.2	Benefits check, lessons learned, closure report	5	22 Jul 2027	28 Jul 2027	1.7.1	0	Toufiq
M7	**Project closed**	0	28 Jul 2027	28 Jul 2027	1.7.2	0	Sponsor

7. Resource allocation and responsibility matrix

Section owner: Kritika Bhattarai

Table 10 assigns responsibility for each major deliverable using RACI: **R**esponsible (does the work), **A**ccountable (approves it; exactly one per row), **C**onsulted and **I**nformed. Assignments were levelled so that no named person and no IAM Architect is booked on two work packages at once (apart from the level-of-effort package 1.1.4). This is why privileged access management (1.3.4) and access review campaigns (1.3.5) follow the MFA policies (1.3.3) instead of running alongside them.

Table 10. RACI matrix

Deliverable (WBS)	Sponsor	PM (Toufiq)	Res. & Comms (Kritika)	QA Lead (Fardin)	Change & Config (Mahir)	IAM Architect	Partner	HR	IT Ops
Project charter and management plan (1.1.1, 1.1.3)	**A**	**R**	C	C	C	C		I	I
Stakeholder register and communication plan (1.1.2)	I	**A**	**R**	I	I	I	I	C	C
Current-state assessment (1.2.1)	I	**A**		C	C	**R**		C	**R**
Requirements and privacy impact assessment (1.2.2–1.2.3)	I	**A**	C	**R**		**R**		C	C
Platform selection and contract (1.2.4, M3)	**A**	**R**	I	**R**	I	C			C
Role model (1.2.5)	I	**A**		C	I	**R**	C	**R**	I
Solution architecture and security design (1.2.6)	I	**A**	I	C	C	**R**	C	I	C
Platform build: JML, MFA, PAM, access reviews (1.3)		I	I	C	I	**A**	**R**	C	C
SSO and provisioning integration (1.4)		I	I	C	I	**A**	**R**		C
Testing: system, penetration, user acceptance (1.5)	I	I	C	**A**	I	C	**R**		C
Training and adoption (1.6.1)	I	I	**A**	C	**R**	C	C	C	**R**
Pilot, rollout and hypercare (1.6.2–1.6.4)	I	**A**	C	C	**R**	C	**R**	I	**R**
Risk register and issue log	I	**A**	**R**	C	C	C	I	I	I
Change control and configuration management	C	**A**	I	C	**R**	C	C		I
Quality assurance reviews and audits	I	**A**	I	**R**	C	C	C		I
Handover and closure report (1.7)	**A**	**R**	C	C	**R**	C	I	I	C

Table 11 estimates labour and other costs. Rates are fully loaded, assume a 38-hour week and exclude GST. The contingency reserve equals the total expected monetary value of the risks in Section 8.

Table 11. Resource and cost estimate (AUD)

Resource or item	Person or source	Allocation	Hours	Rate/h	Cost
Project Manager	Toufiq Ahmed Joy	100% × 39 wks	1,482	115	170,430
Resource and Communications Manager	Kritika Bhattarai	50% × 39 wks	741	90	66,690
Quality Assurance Lead	Fardin Rahman Tanvir	50% × 39 wks	741	90	66,690
Change and Configuration Manager	Ahmed Mahir Daiyan	50% × 39 wks	741	90	66,690
IAM Architect (contract)	To be engaged	80% × 35 wks	1,064	150	159,600
HR systems analyst	People and Culture	20% × 20 wks	152	75	11,400
Service desk and IT operations	IT Operations	50% × 16 wks	304	70	21,280
Implementation partner (fixed-price SOW)					185,000
IAM platform subscription, year 1 (1,500 identities)					162,000
FIDO2 security keys for shared workstations and admins (650)					55,250
Independent penetration test					28,000
UAT backfill shifts for clinical testers					32,000
Training delivery and materials					15,000
Work package estimate					**1,040,030**
Contingency reserve (risk EMV, Section 8)					**159,000**
Cost baseline					**1,199,030**
Management reserve (sponsor)					**50,970**
Budget ceiling					**1,250,000**

8. Risk management plan

Section owner: Toufiq Ahmed Joy

The process follows ISO 31000 [14] and the PMBOK Guide's risk domain [12]:

1. **Identify** risks at kickoff, from the current-state assessment and the research in Section 2, and again at each gate.
2. **Analyse** each risk first by score, using the 5 × 5 scales in Table 12, then in dollars as expected monetary value (EMV = probability × cost impact).
3. **Plan responses**: avoid, mitigate, transfer or accept. Every risk has a mitigation (done beforehand) and a contingency (done if it happens).
4. **Monitor** triggers through each risk's owner. The register is reviewed weekly with clinical and IT staff present [15], top risks go to the sponsor monthly, and the register is version-controlled in risk/risk_register.csv.

Table 12. Probability and impact scales

Score	Probability	Impact (the highest of cost, schedule or clinical effect)
1	Rare (< 10%)	Negligible: < AUD 5,000; < 2 days; no clinical effect
2	Unlikely (10–29%)	Minor: AUD 5,000 to < 20,000; 2–5 days; workaround available
3	Possible (30–49%)	Moderate: AUD 20,000 to < 50,000; 1–2 weeks; disruption at one site
4	Likely (50–69%)	Major: AUD 50,000 to < 100,000; 2–4 weeks; clinical access disrupted at several sites
5	Almost certain (≥ 70%)	Severe: ≥ AUD 100,000; > 4 weeks or a milestone missed; patient-safety effect

Score = probability × impact: 1–4 Low, 5–9 Medium, 10–16 High, 20–25 Extreme.

Table 13. Probability × impact matrix showing where each risk sits

Probability ↓ / Impact →	1 Negligible	2 Minor	3 Moderate	4 Major	5 Severe
5 Almost certain					
4 Likely			**R03**	**R02**	
3 Possible		**R06**	**R05, R07, R10**	**R01, R09**	
2 Unlikely			**R12**	**R04, R11**	**R08**
1 Rare					

Quantitative analysis. Table 14 gives the EMV of the 5 High risks. The EMVs of all 12 risks add up to AUD 158,400, which is rounded up to a contingency reserve of AUD 159,000. The project manager can release contingency only for a risk that has occurred, and records each release in the change log.

Table 14. Expected monetary value of High risks (AUD)

ID	Risk	Probability	Cost impact	EMV
R01	EMR or PAS cannot support modern SSO (SAML/OIDC), needing a custom connector	40%	60,000	24,000
R02	Inaccurate HR data (job codes, managers, end dates) grants wrong access or removes access in error	50%	40,000	20,000
R03	Clinicians resist MFA at shared ward workstations and adopt workarounds	60%	30,000	18,000
R09	Credential phishing attack succeeds during the transition while old and new sign-ins coexist	30%	60,000	18,000
R08	Misconfigured conditional access policy locks users or administrators out	20%	80,000	16,000
	All 12 risks (contingency basis)			**158,400**

The full register is shown in Table 15 on the next page.

Table 15. Risk register (all risks open at baseline; P = probability score, I = impact score, EMV in AUD)

ID	Risk	Category	P	I	Score	EMV	Owner	Response	Mitigation (before)	Contingency (if it happens)	Trigger
R01	EMR or PAS cannot support modern SSO (SAML/OIDC), needing a custom connector	Technical	3	4	**12 High**	24,000	IAM Architect	Mitigate	Confirm protocol support for every in-scope app during the current-state assessment (1.2.1); make vendor-confirmed connectors a mandatory RFQ criterion	Front the app with an application proxy or password-vaulting SSO and move it to wave 3	App vendor cannot confirm SAML/OIDC support by Gate 2
R02	Inaccurate HR data (job codes, managers, end dates) grants wrong access or removes access in error	Data / privacy	4	4	**16 High**	20,000	Kritika (with HR)	Mitigate	HR data cleansing before 1.3.2; run leaver automation in report-only mode for two weeks with daily reconciliation	Switch affected changes to a manual approval queue; restore wrongly removed clinical access through the break-glass process within 15 minutes	Reconciliation mismatch above 2% of records
R03	Clinicians resist MFA at shared ward workstations and adopt workarounds	People	4	3	**12 High**	18,000	Kritika	Mitigate	Tap-and-go FIDO2 security keys for shared workstations; clinical champions at every site; nurses in UAT	Extend that site's rollout by one week with floor-walkers and extra keys	Site MFA enrolment below 80% two weeks after rollout
R04	Implementation partner loses or reassigns a key integration engineer	Vendor	2	4	**8 Medium**	12,500	Toufiq	Transfer	Named-resource clause with 10-day replacement and service credits; configuration documented in GitHub	Call on the standing-offer second partner; resequence SSO waves to protect the critical path	Key engineer absent for more than 5 working days
R05	Platform procurement and contract negotiation run late, delaying design	Schedule	3	3	**9 Medium**	10,000	Toufiq	Mitigate	Issue the RFQ in week 3 with a pre-approved contract template; legal engaged from kickoff	Start role model and test strategy early and use the vendor's trial tenant for design	Contract not signed by the M3 date
R06	Licence prices rise (USD-priced quote or exchange rate) before contract signature	Budget	3	2	**6 Medium**	5,400	Toufiq	Mitigate	Require AUD-denominated pricing held for 90 days in the RFQ	Draw on contingency; license lower-risk contractor identities on a cheaper tier	Final quote exceeds the baseline licence line by more than 5%
R07	Clinical staff unavailable for UAT because of roster pressure	Organisational	3	3	**9 Medium**	8,000	Fardin	Mitigate	Book testers six weeks ahead with funded backfill shifts; agree dates with the Director of Nursing	Add evening UAT sessions and test critical clinical scenarios first; if UAT still slips, shorten the pilot from 10 to 7 days with sponsor approval	Fewer than 70% of booked testers attend in UAT week 1
R08	Misconfigured conditional access policy locks users or administrators out	Technical	2	5	**10 High**	16,000	IAM Architect	Mitigate	Two monitored break-glass accounts excluded from policies; policies run in report-only mode first; every policy change peer-reviewed through change control	Invoke the rollback runbook and break-glass access; communicate through the service desk	More than 20 lockouts in one hour
R09	Credential phishing attack succeeds during the transition while old and new sign-ins coexist	External	3	4	**12 High**	18,000	IAM Architect	Mitigate	MFA for remote access and admin accounts in wave 1; sign-in risk alerts monitored daily	Run the cyber incident response plan; assess for a notifiable data breach to the OAIC	Risky sign-in alert or user report of a suspicious prompt
R10	Departments push extra applications into phase 1 (scope creep)	Organisational	3	3	**9 Medium**	13,500	Mahir	Avoid	Signed scope statement with a named out-of-scope list; all requests through the change control board	CCB defers new apps to the phase 2 backlog; contingency used only with sponsor approval	More than two change requests for new apps in a month
R11	Identity or audit data stored offshore conflicts with privacy obligations	Data / privacy	2	4	**8 Medium**	10,000	Fardin (with Privacy Officer)	Avoid	Australian data residency as a mandatory RFQ criterion; privacy impact assessment (1.2.3) before contract	Select the vendor's Australian region or the runner-up vendor	PIA identifies offshore storage of personal information
R12	Project manager or a workstream lead unavailable for an extended period	People	2	3	**6 Medium**	3,000	Toufiq	Accept	Named deputy for each role; decisions and plans kept in GitHub and Jira	Deputy steps in; re-baseline the schedule through change control if needed	Absence longer than 10 working days

9. Issue management strategy

Section owner: Kritika Bhattarai

A risk *might* happen; an issue *has* happened and needs action now. When a risk occurs, its owner starts the contingency and logs an issue. Table 16 sets out how issues are handled. The log is reviewed weekly, P1 issues are reviewed daily until closed, and open issues go in the monthly sponsor report.

Table 16. Issue handling steps

Step	What happens	Who
1. Log	Record the issue within one working day as a Jira card of type *Issue*; the issue log (templates/issue_log.csv) mirrors it	Anyone
2. Prioritise	Set priority P1–P4 using Table 17	Workstream lead
3. Assign	Name a single owner and a target date	Workstream lead
4. Escalate	If not resolved within the response time, escalate as in the priority table. An issue that needs more time, money or scope becomes a change request (Section 10)	Owner
5. Close	Record the action taken and close. If the issue revealed a new risk, add it to the register	Owner; project manager closes

Table 17. Issue priorities, response times and escalation

Priority	Definition	Owner assigned	Escalate if unresolved
P1 Critical	Stops clinical work, go-live or a gate, or is a security incident	Within 2 hours	Project manager immediately; sponsor within 24 hours
P2 High	Threatens a critical-path task or milestone date	Within 1 working day	Project manager after 2 days; sponsor or CCB after 5 days
P3 Medium	Affects a task that still has float	Within 3 working days	Workstream lead, then project manager after 10 days
P4 Low	Minor; no schedule effect	Next weekly meeting	Project manager at the weekly review

Table 18. Issue log (I-001 and I-002 are examples of how to fill it in; I-003 is a real issue from preparing this plan)

ID	Raised	Description	By	Priority	Owner	Action	Status	Closed
I-001	12 Jan 2027	HR export has no end dates for 41 contractor records, so leaver rules cannot run for them	IAM Architect	P2	Kritika (with HR)	HR backfills end dates; contractor coordinator confirms them	Closed	20 Jan 2027
I-002	2 Mar 2027	Pathology portal vendor's test environment is unavailable for wave 3	Partner	P3	Mahir	Vendor escalation; test against staging copy if not fixed in 5 days	Open	
I-003	25 Oct 2026	Plan pull requests #1–#4 were merged into main without the required review (#4 by admin bypass), and #4 placed files in the repository root instead of plan/sections/	Toufiq	P2	Toufiq	Retrospective review comments on #1–#3; Fardin re-submits #4's files to plan/sections/ through a reviewed pull request; no more admin bypass	Open	

10. Change request and change management process

Section owner: Ahmed Mahir Daiyan

Change control protects the scope, schedule and cost baselines approved at Gate 1. Figure 3 shows the process.

- **Raise:** anyone can submit a change request (CR) on the form in Table 19.
- **Log:** within one working day the Change and Configuration Manager logs it as CR-###.

- **Assess:** within five days the project manager, IAM Architect and QA Lead assess its impact on scope, time, cost, quality and risk.
- **Decide:** the board in Table 20 decides, within the approval limits in the figure.
- **Implement:** approved changes are made on a GitHub branch (cr-###), merged after the QA Lead reviews them, and saved as a new tagged baseline.
- **Close:** every decision is recorded in the change log (Table 21).

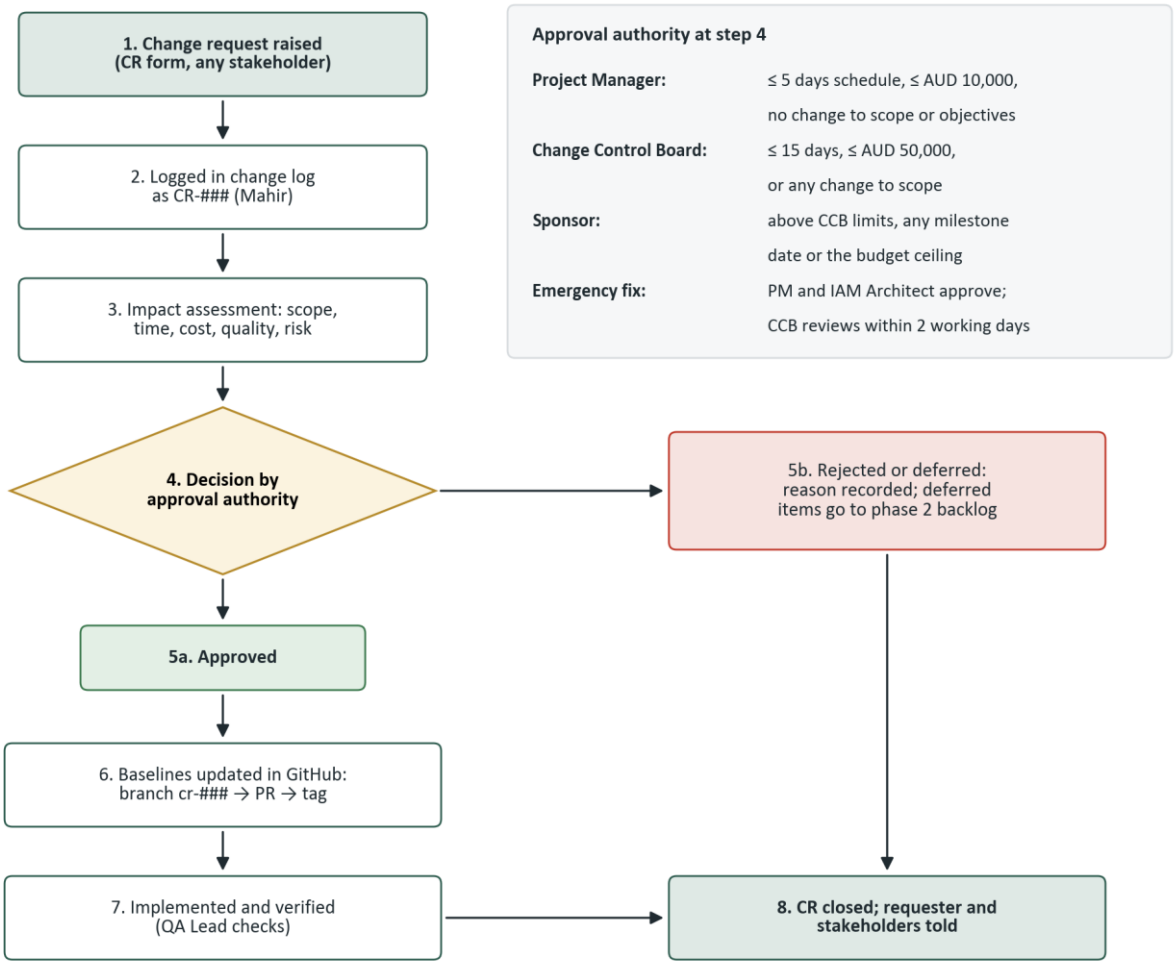


Figure 3. Change control process and approval authority

Table 19. Change request form template

CR ID and date raised	CR-### (assigned by Change and Configuration Manager); dd/mm/yyyy
Requested by	Name, role, department
Description of change	
Reason and benefit	
WBS elements affected	e.g. 1.4.3, M5
Impact: scope	Deliverables added, removed or changed
Impact: schedule	Working days; milestones moved; effect on critical path
Impact: cost	AUD; funding source (contingency, management reserve, new funds)
Impact: quality and risk	Effect on acceptance criteria; new or changed risks (IDs)
Urgency	Normal / urgent / emergency

Recommendation	Project manager's recommendation and options considered
Decision	Approved / rejected / deferred, with reason; approver name and date
Implementation	Owner; GitHub branch and baseline tag; Jira card

Table 20. Change control board (meets fortnightly, or within two working days for an urgent CR)

CCB role	Member	Part in decisions
Chair	Catherine Doyle, Chief Operating Officer (sponsor)	Final approval; casting vote
Members	Project manager (presents CRs); IAM Architect; IT Operations Manager; QA Lead; Director of Nursing's delegate (clinical impact)	Vote
Secretary	Change and Configuration Manager (Mahir)	Keeps the change log; no vote

Table 21. Change log (the two rows are examples of how to fill it in)

ID	Date	Change	Impact	Decision	Approver	Baseline	Status
CR-001	18 Feb 2027	Add the dental practice system to phase 1 SSO	+8 days on wave 3; +AUD 22,000	Deferred to phase 2	CCB	–	Closed
CR-002	1 Mar 2027	Extra night-shift training sessions at the day hospital	0 days (1.6.1 has float); +AUD 3,500 from contingency	Approved	Project manager	baseline-1.1	Closed

11. Configuration management (GitHub)

Section owner: Ahmed Mahir Daiyan

Every project artefact that defines a baseline is a configuration item and is kept under version control in one GitHub repository:

https://github.com/AR13X3/iam-deployment-project-plan

Table 22. Repository structure

Folder or file	Contents	Owner
README.md	Purpose, folder guide, branch and commit rules, how to raise a CR	Mahir
plan/	Plan drafts and released versions (.docx)	Toufiq
schedule/	GanttProject source (IAM_Schedule.gan), schedule.csv, Gantt image	Toufiq
wbs/	WBS outline (wbs.csv) and diagram	Toufiq
risk/	Risk register (risk_register.csv)	Toufiq
change-management/	Change log, CR form and process flowchart	Mahir
templates/	Issue log, status report and CR templates	Kritika
config/ (from build)	Exported IAM platform configuration (access policies, role definitions, provisioning mappings) as JSON	IAM Architect

Table 23. Version-control rules

Practice	Rule
Branches	main is protected. Each owner works on a branch for their area (charter, schedule, risk, change-management, quality, communication) and merges through a pull request approved by another member. Approved change requests use cr-### branches
Commits	One commit per meaningful change, with an imperative message naming the area, e.g. *risk: add EMV for high risks* or *schedule: add SS+5d link for 1.4.4*
Versions	Drafts are tagged v0.x; the released plan is GitHub Release v1.0; later approved baselines are tagged baseline-1.x
Readable history	Risk register, schedule, WBS and logs are kept as CSV alongside the .docx so changes show line by line

Practice	Rule
Status accounting and audit	At each gate the QA Lead checks that the tagged baseline matches the approved documents and the change log

During the build, platform configuration (MFA and conditional access policies, role definitions, provisioning mappings) is exported to config/, so every production change can be traced to an approved CR. Figure 4, Figure 5 and Figure 6 show the repository.

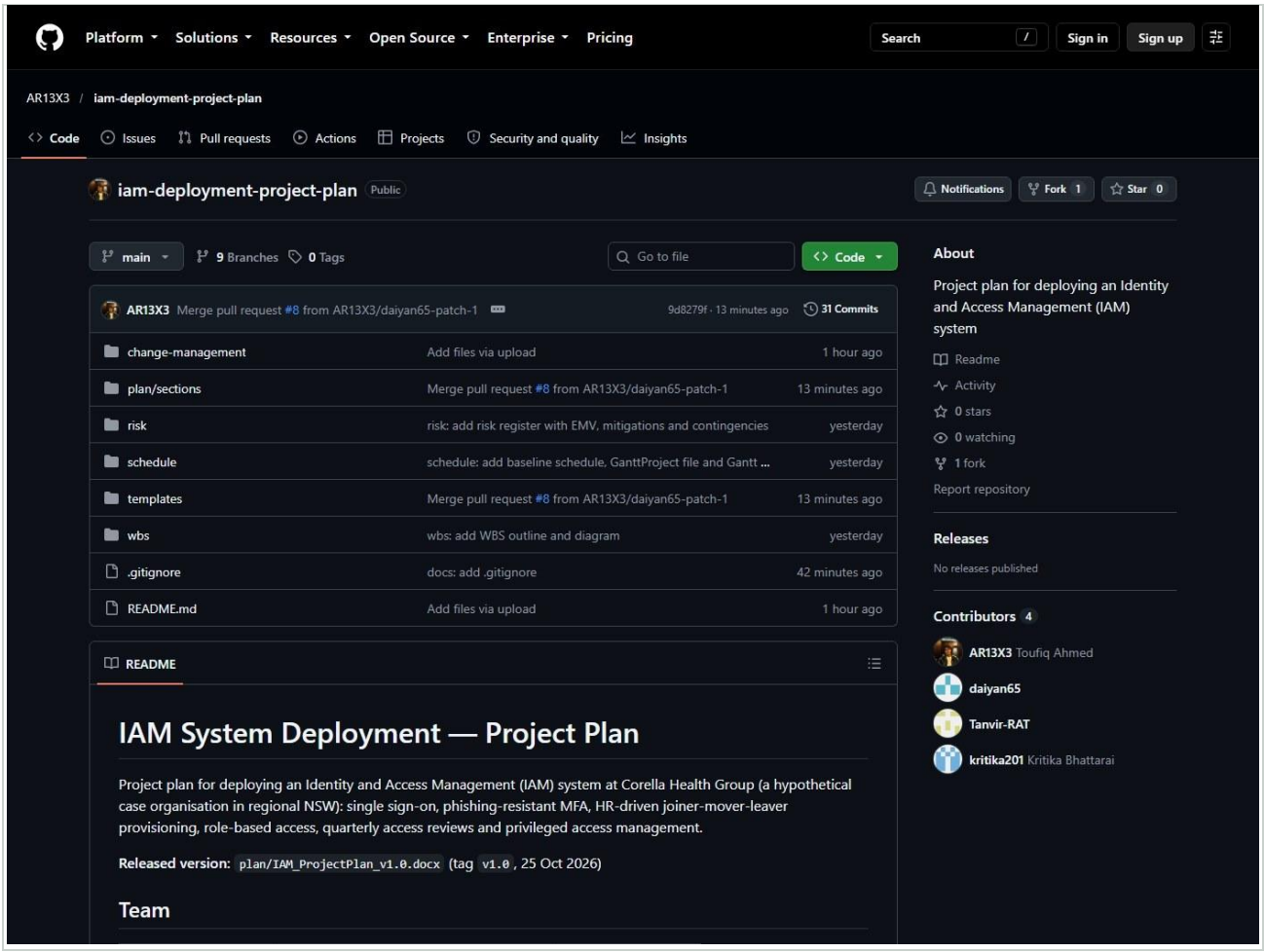


Figure 4. Repository README

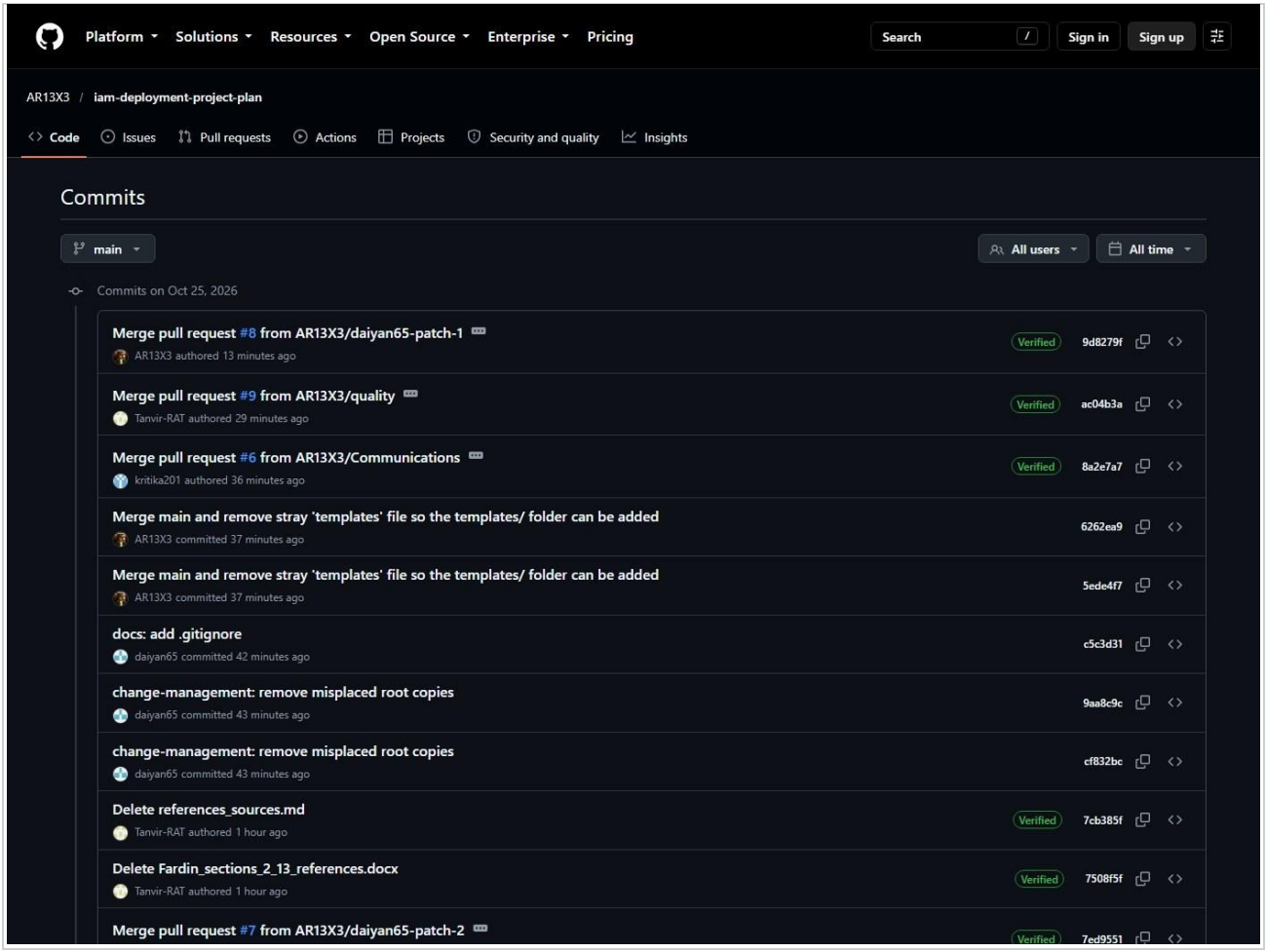


Figure 5. Commit history

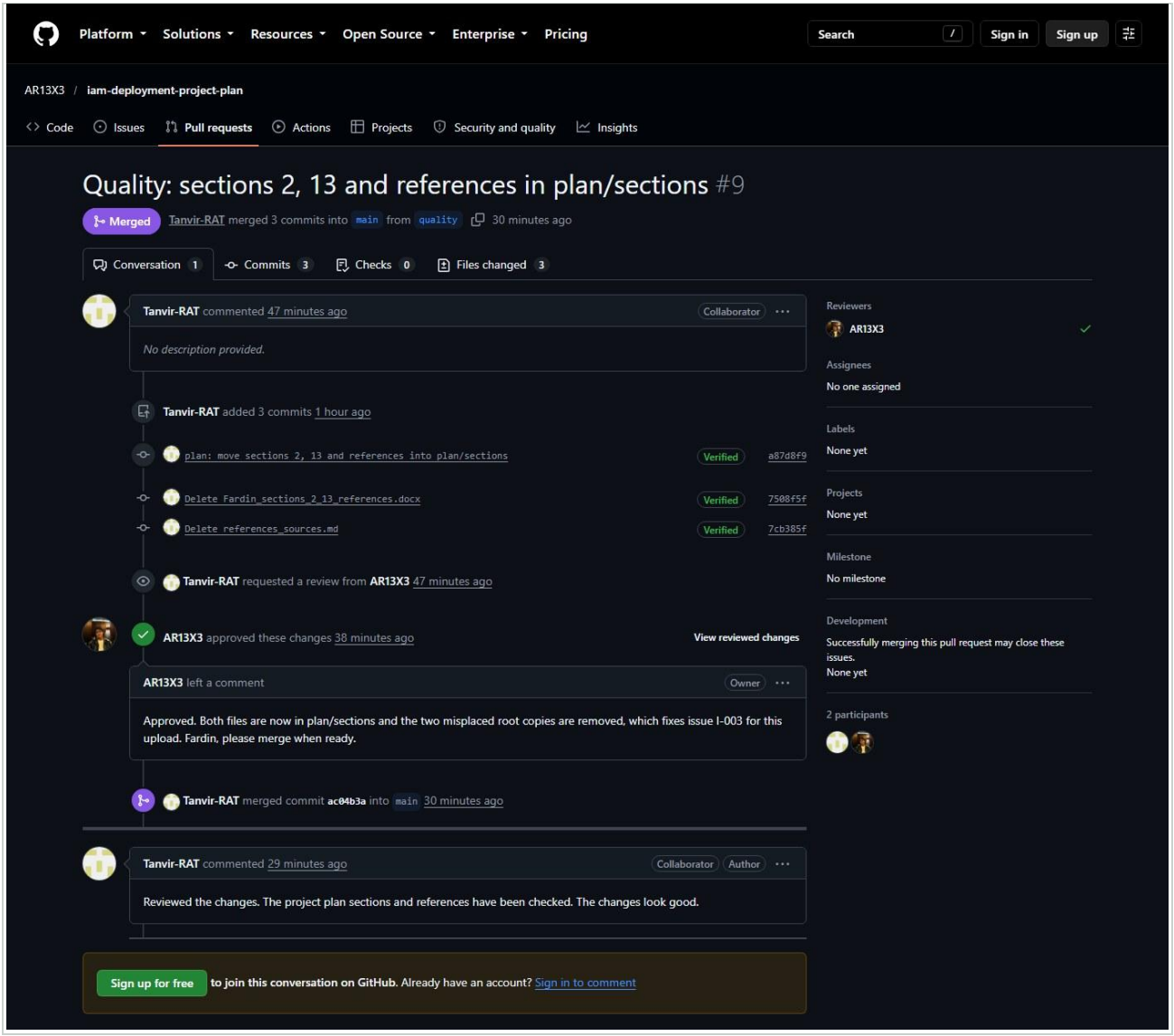


Figure 6. Merged pull request with review

12. Communication management plan

Section owner: Kritika Bhattarai

Table 24 lists every stakeholder from the charter, plus the project team and the CCB. For each it shows what they need to know, how and how often they hear it, and who sends it. The tools are:

- **Microsoft Teams** for meetings: a 15-minute stand-up three times a week and a weekly status meeting;
- **Jira** for tracking tasks;
- **GitHub** for documents and baselines;
- **email and the intranet** for reaching staff.

Problems are escalated as set out in Section 9.

Table 24. Communication plan

Stakeholder	Information needed	Method	Frequency	Owner
Project sponsor	Status against baseline; top risks and issues; decisions needed	One-page status report and 30-minute meeting	Fortnightly; at each gate	Toufiq

Stakeholder	Information needed	Method	Frequency	Owner
Executive leadership team and Board Audit and Risk Committee	Progress to the Essential Eight target; budget; key risks	Written briefing	Monthly (ELT); quarterly (committee)	Toufiq
Head of Information Technology	Design decisions; handover readiness	Steering meeting; design reviews	Fortnightly	Toufiq
IT operations and service desk	Build progress; runbooks; rollout dates; known issues	Teams channel; weekly operations sync; knowledge articles	Weekly; daily during rollout	Mahir
Clinical staff and the Director of Nursing	What changes, when; how to enrol; where to get help	Champions at site huddles; intranet news; quick-reference cards	Monthly; weekly from 4 weeks before their site goes live	Kritika
Administrative and corporate staff	What changes, when; how to enrol	Email; intranet; short videos	Monthly; weekly before rollout	Kritika
Contractors, agency staff and visiting medical officers	Enrolment steps; how to request access	Email through agency coordinators; onboarding pack	At onboarding and before rollout	Kritika
People and Culture (HR)	Data quality issues; changed joiner-mover-leaver process	Working session	Weekly during design and build; then fortnightly	Kritika
Privacy Officer and compliance	Privacy impact assessment; data residency; audit evidence	Meeting and assessment report	During 1.2.3 and at each gate	Fardin
IAM platform vendor and implementation partner	Tasks, defects, change requests	Jira board; weekly delivery meeting	Weekly	Toufiq
Application owners	Integration dates; test windows; cut-over and rollback plans	Email and integration meeting	2 weeks before and during their wave	Mahir
Project team	Tasks, blockers, decisions	Jira; Teams stand-up; GitHub pull requests	Stand-up 3 times a week; weekly status meeting	Toufiq
Change control board	CRs with impact assessments	CR pack and meeting	Fortnightly, or within 2 days if urgent	Mahir

13. Quality management approach

Section owner: Fardin Rahman Tanvir

Standards. Project quality management follows the principles of ISO 9001:2026 [16]. The IAM system is designed to support CHG's information security management system under ISO/IEC 27001:2022 [17], in particular its controls for access control, identity management, authentication information, access rights, privileged access and secure authentication. Authentication follows NIST SP 800-63B-4 [6], and the target is Essential Eight Maturity Level Two [7].

Table 25 separates quality assurance (is the process right?) from quality control (is the product right?). Table 26 sets measurable targets.

Table 25. Quality assurance and quality control activities

Quality assurance (process)	Quality control (product)
• Every deliverable is peer-reviewed through a GitHub pull request before merging • Gate reviews at M2, M4 and M5 check entry and exit criteria • The QA Lead audits the risk register and change log at the midpoint of the build • The partner's configuration is reviewed against the approved design before testing	• System test cases are traced to every requirement • UAT uses real clinical scenarios • Independent penetration test, retested after fixes • Reconciliation reports confirm MFA enrolment and joiner-mover-leaver results • Acceptance criteria (Section 4) checked before sign-off

Table 26. Quality metrics

Metric	Target	Method	When	Owner
Requirements covered by test cases	100%	Traceability matrix	Before system testing	Fardin
System and UAT test cases passed	≥ 95%; no open severity 1–2 defects	Test log	Gate 3	Fardin
Penetration-test findings	No open high or critical findings	Test report and retest	Before M5	IAM Architect
SSO sign-in success rate	≥ 99.5%	Platform sign-in logs	Daily in rollout; then weekly	IT Ops
Phishing-resistant MFA enrolment	100% of in-scope identities	Enrolment report	Weekly in rollout	Mahir
Leaver access removal	≤ 4 hours	Monthly sample of 30 HR terminations against application logs	Monthly from go-live	Fardin
New starters with access on day one	≥ 95%	Joiner report and onboarding survey	Monthly	Kritika
Orphaned accounts in in-scope apps	0	Account reconciliation	End of hypercare; then quarterly	IAM Architect
Privileged accounts	≤ 12, all with FIDO2 keys	PAM report	Go-live; then quarterly	IAM Architect
Deliverables peer-reviewed before merge	100%	GitHub pull request history	Each gate	Mahir
Password-reset tickets	50% fewer than Q1 2027	Service desk reports	3 months after go-live	IT Ops
Staff satisfaction with sign-in	≥ 4 out of 5	Pulse survey	4 weeks after each site goes live	Kritika

14. Conclusion

Section owner: Kritika Bhattarai

This plan turns CHG's audit findings into a funded, scheduled project. Research shows only phishing-resistant MFA meets the Essential Eight benchmark (Section 2). Seven measurable objectives (Section 3) and a charter with 12 in-scope applications and a AUD 1,250,000 ceiling (Section 4) set the boundaries. 29 work packages and seven milestones (Section 5) run 182 working days to 28 Jul 2027, with go-live on 16 Jun 2027 (Section 6).

Each deliverable has one accountable owner within a AUD 1,199,030 cost baseline (Section 7). 12 risks, 5 High, each have a mitigation, contingency and trigger, and their EMV funds the AUD 159,000 contingency (Section 8). Issues follow four priority levels (Section 9); changes pass approval limits and a change control board (Section 10) into tagged GitHub baselines (Section 11). All 13 stakeholder groups are reached on a fixed schedule (Section 12), and 12 quality measures each have a target and owner (Section 13). The next step is the sponsor's approval of the charter (M1).

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Section owner: Fardin Rahman Tanvir

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Appendix A. Jira project evidence

Section owner: Ahmed Mahir Daiyan

The Jira Scrum project holds one epic for the plan and one card per section, each assigned to its owner with a due date. The screenshots below show the board during the sprint, the backlog, a card's comments and history, and the timeline.

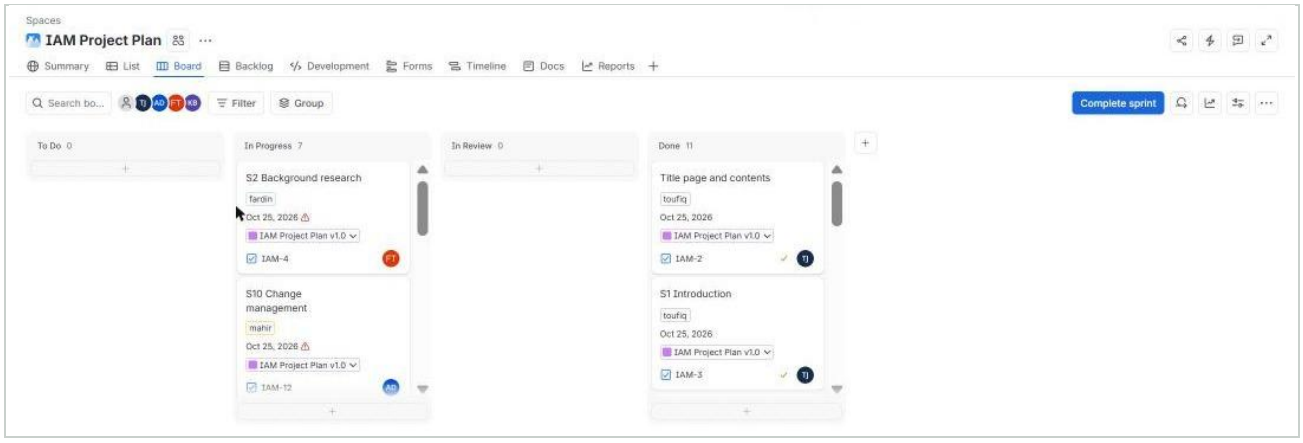


Figure 7. Jira board during the sprint

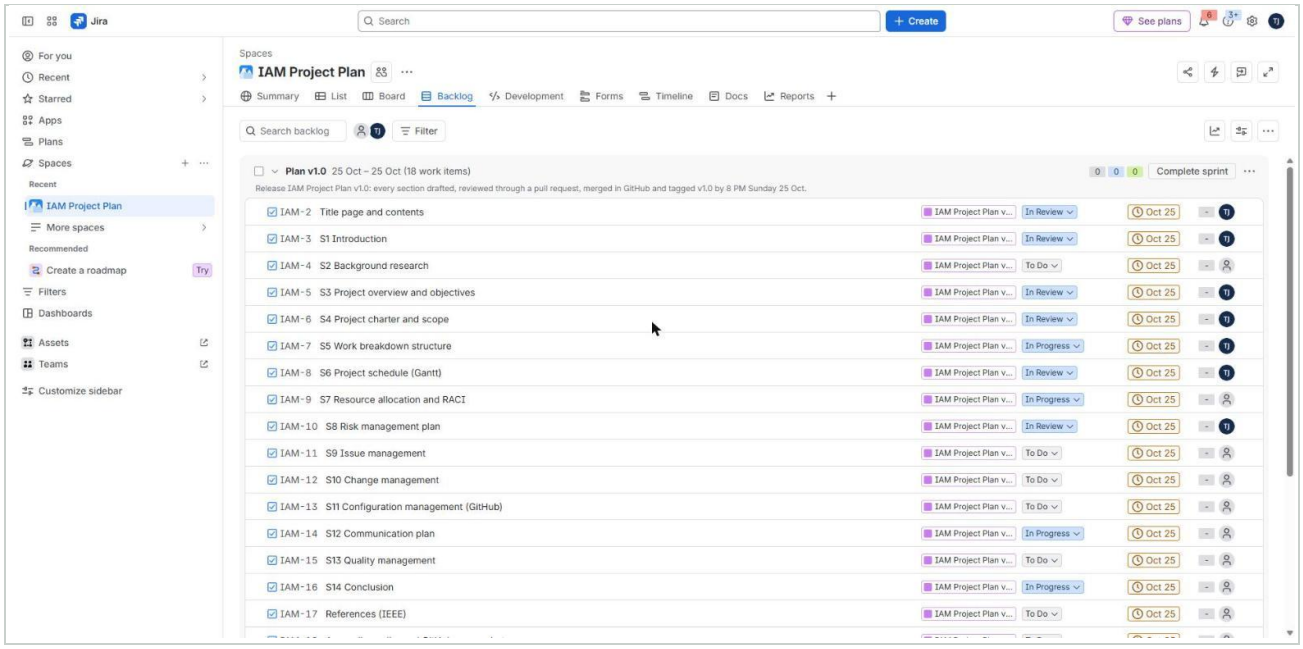


Figure 8. Jira backlog

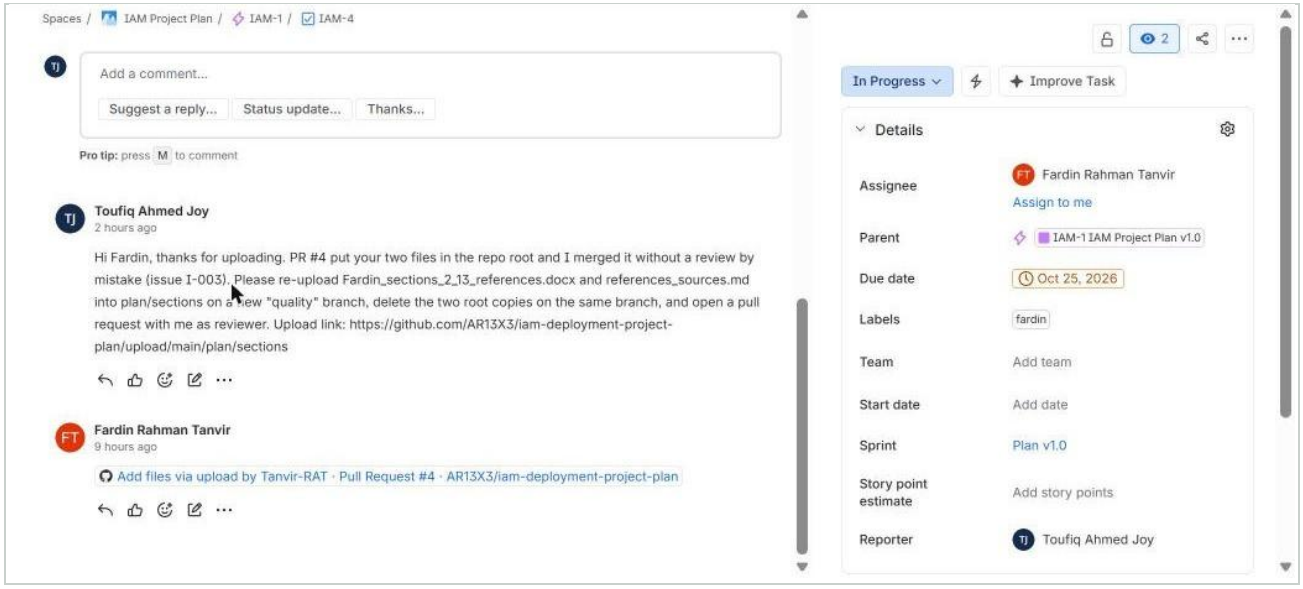


Figure 9. Card comments and history

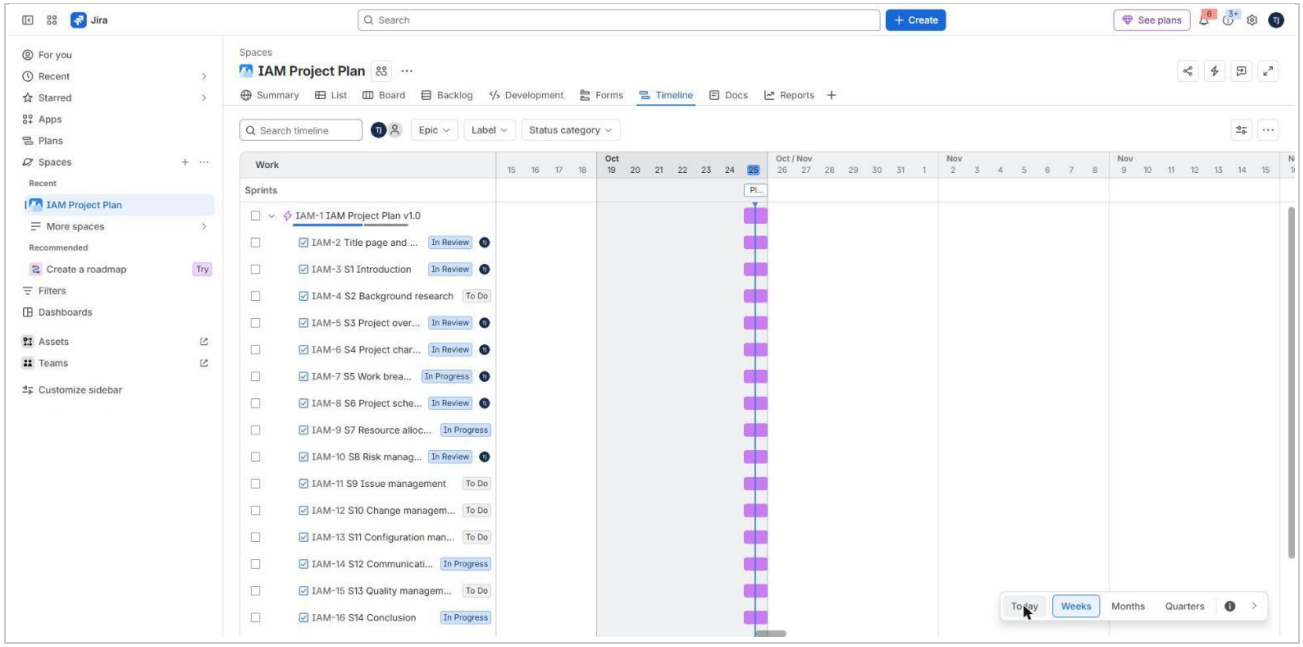


Figure 10. Jira timeline

Appendix B. GitHub history

Section owner: Ahmed Mahir Daiyan

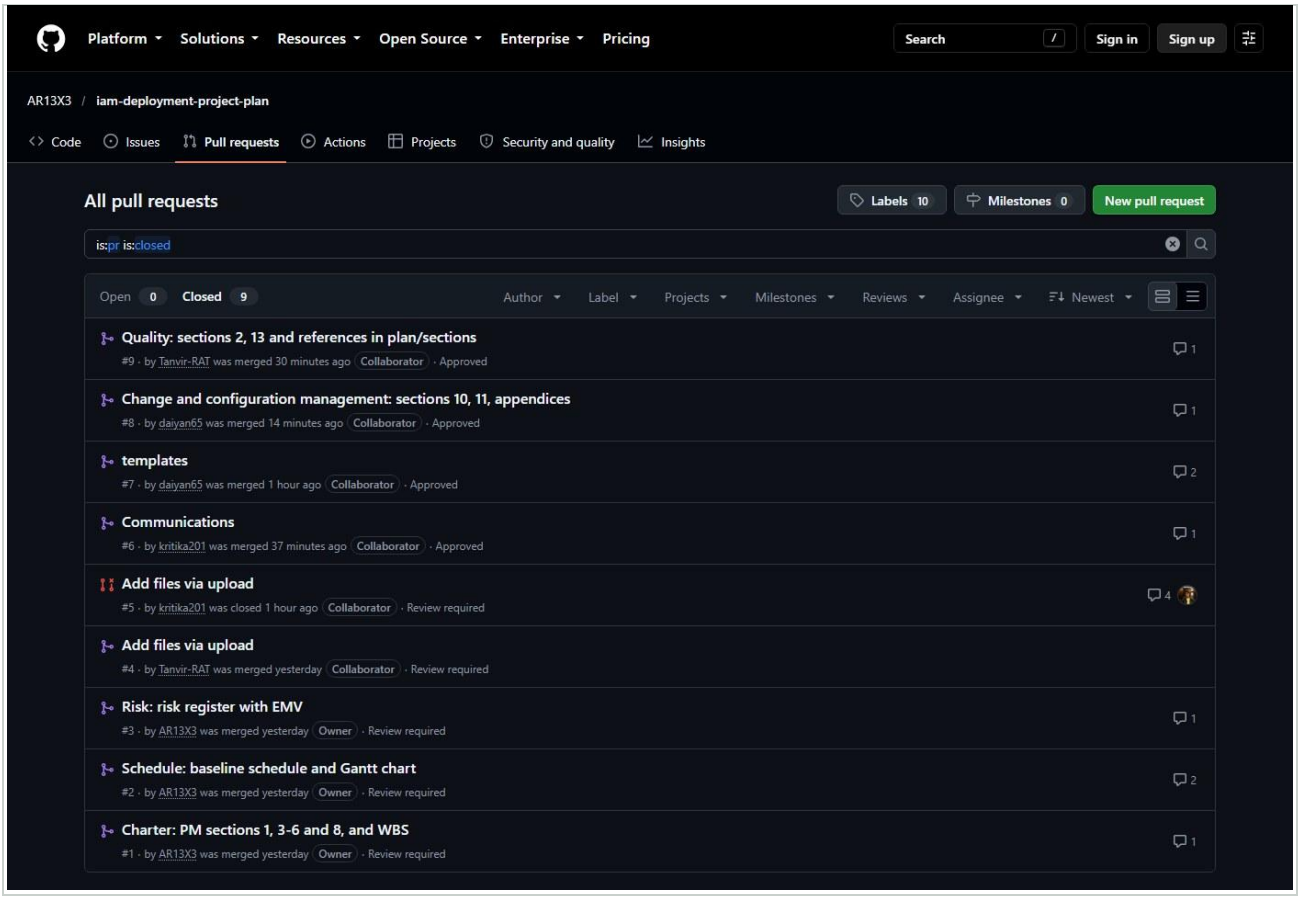


Figure 11. Merged pull requests

Appendix C. Working calendar

Section owner: Ahmed Mahir Daiyan

The schedule uses a Monday-to-Friday calendar with these non-working days. All other weekdays are working days.

Table 27. Non-working weekdays in the schedule

Date	Reason
24 Dec 2026	Christmas shutdown
25 Dec 2026	Christmas Day
28 Dec 2026	Boxing Day (additional day)
29 Dec 2026	Christmas shutdown
30 Dec 2026	Christmas shutdown
31 Dec 2026	Christmas shutdown
1 Jan 2027	New Year's Day
26 Jan 2027	Australia Day
26 Mar 2027	Good Friday
29 Mar 2027	Easter Monday
14 Jun 2027	King's Birthday